2.3 Acute Pain*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain is sharp and does not go away." • Pain score 7/10 **Objective** • Protects the painful area • Restless, grimacing • HR 106/min, RR 22/min	**ACTUAL** Acute pain related to a physical injury agent as evidenced by a pain score of 7/10, guarding and tachycardia.	• Pain at or below 3/10 within 1 hour of intervention. • Able to rest and take part in care.	• Establish the site, intensity, character and duration of pain. • Give the prescribed analgesic and reassess after 30–60 minutes. • Support the affected part in a comfortable position. • Apply cold or warm packs if ordered. • Teach relaxation and paced breathing. • Accept and document the patient's own report of pain.	• Met when pain is 3/10 or lower. • Otherwise, the regimen is reviewed with the physician.
Subjective • "I wake up because of the pain." • Sleeps about 3 hours a night **Objective** • Yawning; dark circles under the eyes • Irritable	**ACTUAL** Ineffective sleep pattern related to pain as evidenced by frequent awakening and reported short sleep duration.	• At least 5 hours of sleep a night within 2 nights.	• Give analgesia at bedtime. • Dim lights and reduce noise overnight. • Group care so that rest is not broken. • Support his usual bedtime routine.	• Achieved if he reports 5–6 hours of sleep. • Poor sleep despite this suggests nighttime pain needs reassessment.
Subjective • No bowel action for 2 days **Objective** • Receiving opioid analgesia • Less mobile than usual	**RISK** Risk for impaired intestinal elimination as evidenced by risk factors: opioid use and reduced physical activity.	• Soft stool passed at least every 2–3 days.	• Chart bowel actions daily. • Encourage 2–2.5 L of fluid a day unless restricted. • Encourage dietary fiber and as much movement as he can manage. • Give the prescribed stool softener or laxative alongside the opioid.	• Outcome reached with a soft stool every 2 days. • Three days without stool, or abdominal distension, is reported.

* **References for this care plan:** [14], [35], [48], [110], [130]. Full entries are listed in the References section of the CPMN book.